

Appendix 1. Audit Worksheets

Electronic Copies of Auditing Worksheets

This edition of the *Auditors' Desk Reference* includes access to formatted copies of the auditing worksheets found in this manual. To access these worksheets go to this address: <https://www.optumcoding.com/ProductUpdates/>. Please use the following password to access updates: **AUDIT25**

Customers are permitted to reproduce these worksheets for use within their own facility or medical practice. Wider licensing of this content is available. Other distribution is prohibited.

These audit worksheets can be used when auditing the different areas of CPT® codes.

Modifier Worksheet

The following worksheet may be used to collect the necessary data when auditing a medical record for modifier use.

Account/medical record number: _____ Date of service: _____											
Reviewer: _____ Date of review: _____											
Type of review: _____											
Documentation											
	Supports Modifier Assignment		Provides Necessary Detail		Authenticated		Comments				
	Yes	No	Yes	No	Yes	No					
Modifier											
Modifier											
Modifier											
Assignment											
	Correct Modifier Assigned		Appended to Correct Code		Valid for Procedure		Guidelines Applied		No Code Describing Service		Comments
	Yes	No	Yes	No	Yes	No	Yes	No	Yes	No	
Modifier											
Modifier											
Modifier											
Reimbursement											
	Fee Revisions Made		Comments								
	Yes	No									
Modifier											
Modifier											
Modifier											
Payer Issues											
	Modifier Processed		Payment Adjustment Made Correctly		Prevent Denial		Comments				
	Yes	No	Yes	No	Yes	No					
Modifier											
Modifier											
Modifier											
Claim Issues											
	Indicated on Claim Correctly		Claim Attachments Submitted			Payer Inquiries Responded To			Comments		
	Yes	No	Yes	No	N/A	Yes	No	N/A			
Modifier											
Modifier											
Modifier											

Evaluation and Management Services Worksheets

Office and Other Outpatient Services Audit Worksheet

The following worksheet may be used to collect the necessary data when auditing a medical record for office and other outpatient services (99202–99205 and 99212–99215). Practices can choose to create a custom audit worksheet for each type of E/M service (e.g., office, hospital inpatient or observation care, nursing facility visit, etc.) as in this example, or they can create one audit worksheet for all E/M services.

Record Number				DOS billed		
Attending			Signed	Yes ☐	No ☐	DOS Rendered
E/M Billed		E/M Documented		E/M Mod Billed		E/M Mod Doc
Incident to:						
When a yes is answered for all of the following, the service may be billed as incident to under Medicare guidelines.					Yes	No
Is the NPP an employee of the practice?					☐	☐
Has the physician seen the patient previously, determined a diagnosis, and documented a plan of care?					☐	☐
Was direct personal supervision by the physician provided for in-office encounters?					☐	☐
Does the physician have an active part in the ongoing care of the patient?					☐	☐
History		Was a medically appropriate history documented?			☐	☐
Examination		Was a medically appropriate exam documented?			☐	☐
Code Assigned		Units		Code Documented		Units
Financial Impact						
Undercoding				Overcoding		
Code		Payment		Code		Payment
				Total Impact on Claim		

Medical Decision Making				
Code	Level of MDM (Based on 2 out of 3 Elements of MDM)	Number and Complexity of Problems Addressed	Amount and/or Complexity of Data to be Reviewed and Analyzed <i>*Each unique test, order, or document contributes to the combination of 2 or combination of 3 in Category 1 below.</i>	Risk of Complications and/or Morbidity or Mortality of Patient Management
☐ 99211	N/A	N/A	N/A	N/A
	Straightforward	Minimal ☐ 1 self-limited or minor problem	☐ Minimal or none	☐ Minimal risk of morbidity from additional diagnostic testing or treatment
	Low	Low ☐ 2 or more self-limited or minor problems; or ☐ 1 stable chronic illness; or ☐ 1 acute, uncomplicated illness or injury; or ☐ 1 stable, acute illness; or ☐ 1 acute, uncomplicated illness or injury requiring hospital inpatient or observation level of care	☐ Limited <i>(Must meet the requirements of at least 1 of the 2 categories)</i> ☐ Category 1: Tests and documents - Any combination of 2 from the following: - Review of prior external note(s) from each unique source* - Review of the result(s) of each unique test* - Ordering of each unique test* or ☐ Category 2: Assessment requiring an independent historian(s) <i>(For the categories of independent interpretation of tests and discussion of management or test interpretation, see moderate or high)</i>	☐ Low risk of morbidity from additional diagnostic testing or treatment
	Moderate	Moderate ☐ 1 or more chronic illnesses with exacerbation, progression, or side effects of treatment; or ☐ 2 or more stable chronic illnesses; or ☐ 1 undiagnosed new problem with uncertain prognosis; or ☐ 1 acute illness with systemic symptoms; or ☐ 1 acute, complicated injury	☐ Moderate <i>(Must meet the requirements of at least 1 out of 3 categories)</i> ☐ Category 1: Tests, documents, or independent historian(s) - Any combination of 3 from the following: - Review of prior external note(s) from each unique source* - Review of the result(s) of each unique test* - Ordering of each unique test* - Assessment requiring an independent historian(s) or ☐ Category 2: Independent interpretation of tests - Independent interpretation of a test performed by another physician/other qualified health care professional (not separately reported) or ☐ Category 3: Discussion of management or test interpretation - Discussion of management or test interpretation with external physician/other qualified health care professional/ appropriate source (not separately reported)	☐ Moderate risk of morbidity from additional diagnostic testing or treatment <i>Examples only:</i> - Prescription drug management - Decision regarding minor surgery with identified patient or procedure risk factors - Decision regarding elective major surgery without identified patient or procedure risk factors - Diagnosis or treatment significantly limited by social determinants of health
	High	High ☐ 1 or more chronic illnesses with severe exacerbation, progression, or side effects of treatment; or ☐ 1 acute or chronic illness or injury that poses a threat to life or bodily function	Extensive <i>(Must meet the requirements of at least 2 out of 3 categories)</i> ☐ Category 1: Tests, documents, or independent historian(s) - Any combination of 3 from the following: - Review of prior external note(s) from each unique source*; - Review of the result(s) of each unique test*; - Ordering of each unique test*; - Assessment requiring an independent historian(s) or ☐ Category 2: Independent interpretation of tests - Independent interpretation of a test performed by another physician/other qualified health care professional (not separately reported) or ☐ Category 3: Discussion of management or test interpretation - Discussion of management or test interpretation with external physician/other qualified health care professional/ appropriate source (not separately reported)	☐ High risk of morbidity from additional diagnostic testing or treatment <i>Examples only:</i> - Drug therapy requiring intensive monitoring for toxicity - Decision regarding elective major surgery with identified patient or procedure risk factors - Decision regarding emergency major surgery - Decision regarding hospitalization or escalation of hospital-level care - Decision not to resuscitate or to de-escalate care because of poor prognosis - Decision regarding parenteral controlled substance

* **Note:** Test panels such as 80047 are counted as a single laboratory test. The differentiation between single or multiple unique tests is defined in accordance with the CPT® code set.

Office and Other Outpatient Services Audit Worksheet – Time Only Reporting

Record Number		DOS billed	
Attending		Signed Yes ☐ No ☐	DOS Rendered
History	Was a medically appropriate history documented?	Yes ☐	No ☐
Examination	Was a medically appropriate exam documented?	Yes ☐	No ☐

The table below shows the time required for each code.

Code	History & Exam	Medical Decision Making	Time in Minutes
99202	Medically appropriate	Straightforward	≥ 15
99203	Medically appropriate	Low level	≥ 30
99204	Medically appropriate	Moderate level	≥ 45
99205	Medically appropriate	High level	≥ 60
99211*	N/A	N/A	N/A
99212	Medically appropriate	Straightforward	≥ 10
99213	Medically appropriate	Low level	≥ 20
99214	Medically appropriate	Moderate level	≥ 30
99215	Medically appropriate	High level	≥ 40

*Physician presence is not required; presenting problems are minimal

The following activities are included in the provider’s time when performed:

- ☐ Preparing to see the patient (e.g., review of tests)
- ☐ Performing a medically appropriate examination and/or evaluation
- ☐ Care coordination (not reported separately)
- ☐ Counseling and educating the patient/family/caregiver
- ☐ Documenting clinical information in the electronic or other health record
- ☐ Independently interpreting results (not reported separately) and communicating results to the patient/family/caregiver
- ☐ Obtaining and/or reviewing separately obtained history
- ☐ Ordering medications, tests, or procedures
- ☐ Referring and communicating with other health care professionals

Was code 99417 reported for prolonged services? Yes ☐ No ☐

The table below shows the time required to report each unit of 99417 in addition to 99205 or 99215.

New Patient		Code	
60–74 minutes		99205	
75–89 minutes		99205 x1 and 99417 x1	
90–104 minutes		99205 x1 and 99417 x2	
105 or more minutes		99205 x1 and 99417 x3 or more for each additional 15 minutes	
Established Patient		Code	
40–54 minutes		99215	
55–69 minutes		99215 x1 and 99417 x1	
70–84 minutes		99215 x1 and 99417 x2	
85 or more minutes		99215 x1 and 99417 x3 or more for each additional 15 minutes	
Code Assigned	Units	Code Documented	Units
Financial Impact			
Undercoding		Overcoding	
Code	Payment	Code	Payment
		Total Impact on Claim	

Note: When reporting prolonged services with codes 99205 or 99215 for Medicare beneficiaries, HCPCS Level II code G2212 should be reported.

Critical Care Audit Worksheet

Patient's name: _____

Patient's medical record number: _____

Date of service: _____

Date of review: _____

Reviewer: _____

Type of review: ☐ Prepayment ☐ Postpayment

Reason for encounter: _____

Type of vital organ failure:

☐ Central nervous system failure

☐ Shock

☐ Hepatic

☐ Respiratory

☐ Circulatory failure

☐ Renal

☐ Metabolic

Were any additional services documented? (This time is included in the total critical care time.)

☐ 36000 Introduction of needle or intracatheter, vein

☐ 36410 Venipuncture, age 3 years or older, necessitating physician's skill or other qualified health care professional (separate procedure), for diagnostic or therapeutic purposes

☐ 36415 Collection of venous blood by venipuncture

☐ 36591 Collection of blood specimen from a completely implantable venous access device

☐ 36600 Arterial puncture, withdrawal of blood for diagnosis

☐ 43752 Naso- or oro-gastric tube placement, requiring physician's skill and fluoroscopic guidance

☐ 43753 Gastric intubation and aspiration(s) therapeutic, necessitating physician's skill (e.g., for gastrointestinal hemorrhage), including lavage if performed

☐ 71045 Radiologic examination, chest; single view

☐ 71046 Radiologic examination, chest; 2 views

☐ 92953 Temporary transcutaneous pacing

☐ 93598 Cardiac output measurement(s), thermodilution or other indicator dilution method, performed during cardiac catheterization for the evaluation of congenital heart defects (List separately in addition to code for primary procedure)

☐ 94002 Ventilation assist and management, initiation of pressure or volume preset ventilators for assisted or controlled breathing; hospital inpatient/observation, initial day

☐ 94003 Ventilation assist and management, initiation of pressure or volume preset ventilators for assisted or controlled breathing; hospital inpatient/observation, each subsequent day

☐ 94004 Ventilation assist and management, initiation of pressure or volume preset ventilators for assisted or controlled breathing; nursing facility, per day

☐ 94660 Continuous positive airway pressure ventilation (CPAP), initiation and management

☐ 94662 Continuous negative pressure ventilation (CNP), initiation and management

☐ 94760 Noninvasive ear or pulse oximetry for oxygen saturation; single determination

☐ 94761 Noninvasive ear or pulse oximetry for oxygen saturation; multiple determinations (e.g., during exercise)

☐ 94762 Noninvasive ear or pulse oximetry for oxygen saturation; by continuous overnight monitoring

Time spent in Critical Care	Start Time	End Time	Total Time	Was provider in constant attendance when critical care was provided?	
				Yes ☐ No ☐ If no, critical care codes should not be reported.	
Code Assigned	Units	Code Documented	Units		Comments
99291					
99292					
Additional Procedures (not included in bundled list)	Code Assigned	Code Documented	Modifier Assigned	Modifier Documented	Comments
Financial Impact					
Undercoding			Overcoding		
Code	Payment		Code	Payment	
			Total Impact on Claim		

Surgical Auditing Worksheet

Account/medical record number: _____

Date of service: _____

Reviewer: _____

Date of review: _____

Type of review: _____

CPT® Code Assignment										
Procedure		Code Assigned	Code Documented	Modifier Assigned	Modifier Documented	Comments				
Place of Service						Number of Units				
Indicated on Claim			Documented			Indicated on Claim			Documented	
Billable Supplies						Nonbillable Supplies				
Undercoding			Overcoding			Undercoding			Overcoding	
Code	Payment		Code	Payment		Code	Payment		Code	Payment
					Total Impact on Claim					

Radiology Auditing Worksheet

The following worksheet may be used to collect the necessary data when auditing radiology services.

Account/medical record number: _____		Date of service: _____			
Reviewer: _____		Date of review: _____			
Type of review: _____					
CPT® Code Assignment					
Procedure	Code Assignment	Code Documented	Modifier Assigned	Modifier Documented	Comments
Place of Service			Number of Units		
Indicated on Claim	Documented	Indicated on Claim	Documented		
Billable Supplies			Nonbillable Supplies		
Provided		Provided			
Billed		Billed			
Financial Impact					
Undercoding			Overcoding		
Code	Payment	Code	Payment		
		Total Impact on Claim			

Laboratory Auditing Worksheet

The following worksheet may be used when auditing laboratory services.

Account/medical record number: _____		Date of service: _____				
Reviewer: _____		Date of review: _____				
Type of review: _____						
	Yes	No				
Order for lab test in chart						
Result of lab test						
CPT® Code						
CPT Code Assigned	Unbundled or Exploded?		Corrected CPT Code			
	Yes	No				
Modifier Assignment						
CPT Code	Modifier Assigned	Appended to correct code	Valid for Procedure	Guidelines Applied	Comments	
Medical Necessity						
	Yes	No	Yes	No	Yes	No
Condition Documented						
ICD-10-CM Code Assigned Correctly						
Corrected ICD-10-CM Code						
Completed and signed ABN						
Units						
Number of Units Performed		Number of Units Billed		Corrected Number of Units		
Financial Impact						
Revised CPT Codes						
Revised Modifier Assignment						
Revised Units						
Total						

Medicine Auditing Worksheet

Account/medical record number: _____ Date of service: _____

Reviewer: _____ Date of review: _____

Type of review: _____

Immunizations/Vaccines/Toxoids — CPT® Code Assignment						
Substance	Code Assignment	Code Documented	Face-to-Face Counseling Documented	E/M Service Documented	E/M Service Billed	Comments

Therapeutic/Prophylactic Injections — CPT Code Assignment						
Substance	Code Assignment	Code Documented	Face-to-Face Counseling Documented	E/M Service Documented	E/M Service Billed	HCPCS Level II Code Reported

Number of Units		
Billed	Documented	Comments

Other Medical Services — CPT Code Assignment					
Procedure	Code Assignment	Code Documented	Modifier Assigned	Modifier Documented	Comments

Place of Service		Number of Units	
Indicated on Claim	Documented	Indicated on Claim	Documented

Billable Supplies				Nonbillable Supplies			
Undercoding		Overcoding		Undercoding		Overcoding	
Code	Payment	Code	Payment	Code	Payment	Code	Payment
				Total Impact on Claim			

Non-Chemotherapy Injections and Infusion Auditing Worksheet

Account/medical record number: _____ Date of service: _____

Reviewer: _____ Date of review: _____

Type of review: _____

Diagnosis		
Documented	ICD-10-CM	Reported Code

Therapeutic, prophylactic or diagnostic injection

☐ Subcutaneous (96372)

☐ Intramuscular (96372)

☐ Intra-arterial (96373)

Substance administered: _____

HCPCS code: _____ HCPCS code reported: _____

The number of units documented: _____ Number of units billed: _____

Push/Drip 15 minutes or less

Time if documented

☐ Intra-arterial (96373)

Substance administered: _____

HCPCS code: _____ HCPCS code reported: _____

Number of units documented: _____ Number of units billed: _____

☐ IV Push, initial or single substance (96374)

Substance administered: _____

HCPCS code: _____ HCPCS code reported: _____

Number of units documented: _____ Number of units billed: _____

☐ Each additions push of new substance (96375)

Substance administered: _____

HCPCS code: _____ HCPCS code reported: _____

Number of units documented: _____ Number of units billed: _____

Substance administered: _____

HCPCS code: _____ HCPCS code reported: _____

Number of units documented: _____ Number of units billed: _____

Substance administered: _____

HCPCS code: _____ HCPCS code reported: _____

Number of units documented: _____ Number of units billed: _____

IV Infusion

☐ Up to one hour (96365)

Substance administered: _____

HCPCS code: _____ HCPCS code reported: _____

Number of units documented: _____ Number of units billed: _____

☐ Each additional hour (96366)

Substance administered: _____

HCPCS code: _____ HCPCS code reported: _____

IV Infusion (continued)	
Number of units documented:	Number of units billed:
☐ Each new drug/substance (96367)	
Substance administered:	
HCPCS code:	HCPCS code reported:
Number of units documented:	Number of units billed:
☐ Concurrent infusion (96368)	
Substance administered:	
HCPCS code:	HCPCS code reported:
Number of units documented:	Number of units billed:
Subcutaneous Infusion	
☐ Up to one hour (96369)	
Substance administered:	
HCPCS code:	HCPCS code reported:
Number of units documented:	Number of units billed:
☐ Each additional hour (96370)	
Substance administered:	
HCPCS code:	HCPCS code reported:
Number of units documented:	Number of units billed:
☐ Additional pump set-up (96371)	

Fracture Care Audit Worksheet

Account/medical record number: _____		Date of service: _____	
Reviewer: _____		Date of review: _____	
Type of review: _____			
Anatomical site: _____		Laterality: _____	
Type of Fracture			
☐ Closed	☐ Spiral	☐ Nondisplaced	
☐ Transverse	☐ Impacted	☐ Nonunion	
☐ Comminuted	☐ Oblique	☐ Routine	
☐ Greenstick	☐ Displaced	☐ Malunion	
☐ Open	☐ Delayed Healing		
How Injury Occurred			
☐ Fall ☐ From ladder ☐ From bike		☐ Playing Sports	
☐ Auto Accident			
Sequela Documented	☐ Yes	☐ No	
Treatment Type			
☐ Open Treatment	☐ Skin Traction	☐ Grafting	
☐ Closed Treatment	☐ Internal Fixation	☐ Re-reduction	
☐ Manipulation	☐ External Fixation	☐ Casting/Splinting	
☐ Skeletal Traction	☐ Percutaneous Fixation		
Supplies			
Documented:	HCPCS Code:		
Code Reported:	Payer Allows Reporting	☐ Yes	☐ No
Diagnosis			
Documented	ICD-10-CM	Reported Code	
Procedure			
Documented	CPT®/HCPCS Code	Reported Code	

Note: See modifier worksheets to determine if CPT modifier is required.

Wound Repair Audit Worksheet

Anatomical site (When multiple wounds are repaired, list each)		
Size of wound (When multiple wounds are repaired, list each)		
Is the wound badly contaminated? ☐ Yes ☐ No		
How Injury Occurred:		
☐ Fall	From _____	
☐ Auto Accident	Type of sport _____	
☐ Play Sports	Indicate object _____	
☐ Accidental Cutting		
Closure: ☐ Single ☐ Layered ☐ Graft ☐ Flap		
Any complicating factors? ☐ Yes ☐ No		
If you answered yes above, please list factors:		
How closure was accomplished: ☐ Glue ☐ Suture ☐ Layered closure		
Note: When chemical cauterization, electrocauterization, or adhesive strips are the only material used for wound closure, the service is included in the appropriate E/M code.		
Supplies		
Documented ☐ Yes ☐ No		
HCPCS Code: _____ Code Reported: _____		
Payer Allows Reporting ☐ Yes ☐ No		
Diagnosis		
Documented	ICD-10-CM	Reported Code
Procedure		
Documented	CPT®/HCPCS Code	Reported Code

Note: See modifier worksheets to determine if CPT modifier is required.

Lower Endoscopy Auditing Tool

Patient’s Name: _____

Patient’s Medical Record Number: _____

Date of Service: _____

Preoperative Diagnosis: _____

Procedure Performed: _____

Findings: _____

Postoperative _____

Procedure Performed: _____

☐ Diagnostic ☐ Screening

Surgical with:

☐ Removal of foreign body-Location _____
Substance injected: _____

☐ With biopsy(ies)-Location _____
Substance injected: _____

☐ With directed submucosal injections-Location _____
Substance injected: _____

Were any of the following performed:

☐ With control of bleeding (unrelated to surgical procedures)

☐ With removal of foreign body-Location _____

☐ With decompression of volvulus-Location _____

☐ With ablation of lesions at-Location _____

☐ With removal of lesion by:

☐ Snare -Location _____

☐ Hot biopsy -Location _____

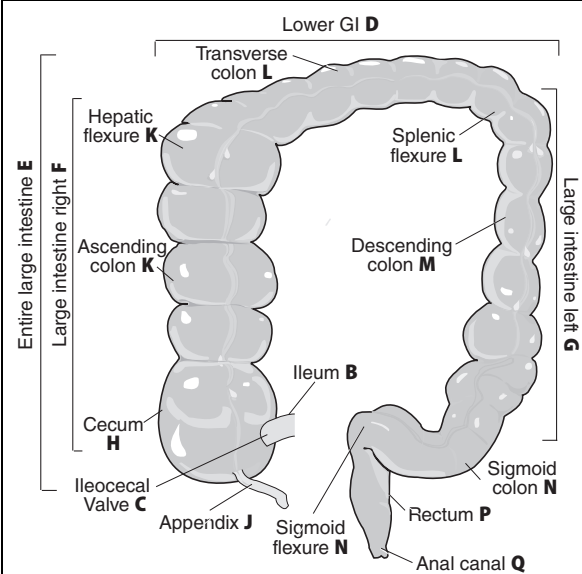
☐ Bipolar Location _____

☐ Dilatation by balloon - Location _____ size of balloon _____ mm

☐ Stent Placement -Location _____

☐ Endoscopic ultrasound -Location _____

☐ Ultrasound guided intramural or transmural FNA/Bx -Location _____



Procedure	Code Assigned	Code Documented	Modifier Assigned	Modifier Documented	Comments
Number of Units					
Indicated on Claim		Documented			
Financial Impact					
Undercoding				Overcoding	
Code		Payment		Code	Payment
				Total Impact on Claim	

Upper Endoscopy Auditing Tool

Patient's Name: _____Patient's Medical Record Number: _____

Date of Service: _____

Preoperative Diagnosis: _____

Procedure Performed: _____

Findings: _____

Postoperative _____

Procedure Performed: _____☐ Diagnostic ☐ Screening

What kind of scope was the procedure performed with:
☐ Flexible, transnasal☐ Flexible, transoral☐ Rigid, transoral

Were any of the following performed:
☐ Biopsy(ies)-Location _____
☐ Ablation of lesions-Location _____
☐ Removal of lesion by: `
☐ Snare-Location _____
☐ Hot biopsy-Location _____
☐ Band ligation of esophageal varices-Location _____
☐ Control of bleeding (unrelated to surgical procedures)-Location _____
☐ Decompression of volvulus-Location _____
☐ Dilation by balloon-Location _____size of balloon _____mm
☐ Over a guidewire-Location _____
☐ Transendoscopic-Location _____

☐ Diverticulectomy of hypopharynx or cervical esophagus
☐ Removal of foreign body
☐ Injections Substance injected: _____
☐ Directed submucosal injections-Location _____
☐ Injection of esophageal varices-Location _____
☐ Stent Placement -Location _____
☐ Endoscopic ultrasound -Location _____
☐ With optical endomicroscopy -Location _____
☐ Ultrasound guided intramural or transmural FNA/Bx -Location _____

Procedure	Code Assigned	Code Documented	Modifier Assigned	Modifier Documented	Comments
Number of Units					Comments
Indicated on Claim		Documented			
Financial Impact					
Undercoding			Overcoding		
Code	Payment		Code	Payment	
			Total Impact on Claim		

Facet Joint Injection Audit Tool

Patient's Name: _____ Patient's Medical Record Number: _____

Date of Service: _____

Name of Provider: _____

Level injected:
☐ Level 1 Unilateral/Bilateral
☐ Level 2 Unilateral/Bilateral
☐ Level 3 Unilateral/Bilateral

Substance Administered: _____

Area: ☐ Lumbar ☐ Thoracic ☐ Cervical

Procedure	Code Assigned	Code Documented	Modifier Assigned	Modifier Documented	Comments

Number of Units		
Indicated on Claim	Documented	Comments

Billable Supplies				Nonbillable Supplies			
Undercoding		Overcoding		Undercoding		Overcoding	
Code	Payment	Code	Payment	Code	Payment	Code	Payment
					-		
				Total Impact on Claim			